

Urinary Incontinence

Outlines:

- Definition of urinary incontinence in the elderly.
 - Causes and risk factors of urinary incontinence in the elderly.
 - Types of urinary incontinence in the elderly.
 - Complication of urinary incontinence in the elderly.
 - The role of Gerontological health nurses for older adults with urinary incontinence
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Definition: (UI):

Is any involuntary leakage of urine.

- It is a common and distressing problem, which may have a profound impact on quality of life.
- Urinary incontinence almost always results from an underlying treatable medical condition but is under-reported to medical practitioners.
- Although it occurs more often as people get older, urinary incontinence isn't an inevitable consequence of aging.

Causes of urinary incontinence:

1. Polyuria (excessive urine production) of which, in turn, the most frequent causes are:
 - Uncontrolled diabetes mellitus.
 - Primary polydipsia (excessive fluid drinking).
 - Central diabetes insipidus.
 - Nephrogenic diabetes insipidus
2. Urinary tract infection: infections can irritate your bladder, causing you to have strong urges to urinate, and sometimes incontinence.
3. Constipation: the rectum is located near the bladder and shares many of the same nerves. Hard, compacted stool in your rectum causes these nerves to be overactive and increase urinary frequency.
4. Obstruction. A tumor anywhere along your urinary tract can block the normal flow of urine, leading to overflow incontinence. Urinary stones sometimes cause urine leakage
5. Certain drinks, foods and medications may act as diuretics —stimulate the bladder and increasing volume of urine such as caffeine or cola beverages, chocolate, heart and blood pressure medications.

6. Enlarged prostate is the most common cause of incontinence in men after the age of 40.
7. Brain disorders: can interfere with nerve signals involved in bladder control, causing urinary incontinence.
 - Multiple sclerosis.
 - Parkinson's disease.
 - Strokes
 - Spinal cord injury

Risk factors of urinary incontinence:

Factors that increase your risk of developing urinary incontinence include:

- **Gender.** Women are more likely to have stress incontinence. Pregnancy, childbirth, menopause and normal female anatomy account for this difference. However, men with prostate gland problems are at increased risk of urge and overflow incontinence.
- **Age.** As you get older, the muscles in your bladder and urethra lose some of their strength. Changes with age reduce how much your bladder can hold.
- **Family history.** If a close family member has urinary incontinence, especially urge incontinence, the risk of developing the condition is higher
- **Being overweight.** Extra weight increases pressure on your bladder and surrounding muscles, which weakens them and allows urine to leak out when you cough or sneeze.
- **Smoking.** This can lead to a chronic cough, which may result in episodes of incontinence.

Types of urinary incontinence UI:

1. **Stress incontinence: (effort incontinence):** is due to insufficient strength of the pelvic floor muscles. Urine leaks when you exert pressure on your bladder by coughing, sneezing, laughing, exercising or lifting something heavy.
2. **Urge incontinence (Overactive bladder):** is a strong urge to urinate even when the bladder isn't full. Urge incontinence may be caused by a minor condition, such as infection, or a more-severe condition such as a neurologic disorder or diabetes.
3. **Mixed urinary incontinence:** usually defined as a combination of Stress UI and Urge UI
4. **Overflow incontinence:** is a urine leakage, with or without feeling a need to go due to a bladder that doesn't empty completely.
5. **Structural incontinence:**
Structural problems can cause incontinence, for example an ectopic ureter, or Fistulas
6. **Functional incontinence:** occurs when a person recognizes the need to urinate, but cannot physically make it to the bathroom in time due to limited mobility.
7. **Bedwetting is episodic UI while asleep.**
8. **Transient incontinence:** is a temporary version of incontinence.
It can be triggered by: - Medications, or severe constipation.

Complications of urinary incontinence

Complications of chronic urinary incontinence include:

- **Skin problems.** Rashes, skin infections and sores can develop from constantly wet skin.
- **Urinary tract infections.** Incontinence increases your risk of repeated urinary tract infections. Long-term use of a urinary catheter significantly increases the risk of infection.

- **Impacts on your personal life.** Urinary incontinence can affect your social, work and personal relationships.

Gerontological Nursing intervention:

A. General principles that apply to prevention and management of all forms of urinary incontinence:

1. Identify and treat causes of transient urinary incontinence.
2. Identify and continue successful pre hospital management strategies for established urinary incontinence.
3. Develop an individualized plan of care using data obtained from the history and physical examination, and in collaboration with other team members. Implement toileting programs as needed.
4. Avoid medications that may contribute to urinary incontinence.
5. Avoid indwelling urinary catheters whenever possible to avoid risk for urinary tract infection.
6. Monitor fluid intake and maintain an appropriate hydration schedule.
7. Limit dietary bladder irritants.
8. Consider adding weight loss as a long-term goal in discharge planning for those with a body mass index (BMI) greater than 27.
9. Modify the environment to facilitate continence.
10. Prevent skin breakdown by providing immediate cleansing after an incontinent episode and utilizing barrier ointments.

B. Strategies for specific problems:

Stress urinary incontinence:

1. Teach pelvic floor muscle exercises.
2. Provide toileting assistance and bladder training (whenever necessary).

3. Consider referral to other team members if pharmacological or surgical therapies are warranted.

Urge urinary incontinence:

1. Implement bladder training.
2. If patient is cognitively intact and is motivated, provide information on urge inhibition.
3. Teach pelvic floor muscle exercises to be used in conjunction with bladder training, and instruct in urge inhibition strategies.
4. Collaborate with prescribing team members if pharmacologic therapy is warranted.
5. Initiate referrals for those patients who do not respond to the previous steps.

Overflow urinary incontinence:

1. Allow sufficient time for voiding.
2. Discuss with interdisciplinary team the need for determining a post-void residual.
3. If catheterization is necessary, sterile intermittent is preferred over indwelling catheterization.
4. Initiate referrals to other team members for those patients requiring pharmacological or surgical intervention.

Functional urinary incontinence:

1. Provide individualized, scheduled toileting, timed voiding, or prompted voiding.
2. Provide adequate fluid intake.
3. Refer for physical and occupational therapy.
4. Modify environment to maximize independence with continence.

I. Nursing Assessment of Urinary Incontinence

Include the following steps:

- **Comprehensive history:** onset, frequency, volume, triggers, fluid intake patterns, associated symptoms.
- **Bladder diary:** patient logs voiding times, volumes, leakage episodes, and fluid intake for 3–7 days.
- **Physical examination:** abdominal, pelvic, rectal, and neurological assessments.
- **Urinalysis and culture:** to rule out infection.
- **Post-void residual measurement:** using bladder scanner or catheter.

II. Patient Education Topics

- Teach **how to keep a bladder diary**.
- Provide instruction on **pelvic floor exercises (Kegels)** with visuals.
- Explain **dietary triggers** (e.g., caffeine, alcohol, artificial sweeteners).
- Clarify when to **seek medical evaluation** (e.g., new onset incontinence, blood in urine, pain).
- Reassure that incontinence is **treatable and not a normal part of aging**.

III. Multidisciplinary Collaboration

- Work with:
 - **Physicians/Urologists:** diagnosis and treatment planning.
 - **Dietitians:** to modify fluid and diet intake.
 - **Physiotherapists:** pelvic floor training and mobility support.
 - **Occupational therapists:** toileting aids, bathroom modifications.
 - **Social workers:** help with financial or psychosocial concerns.
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IV. Assistive Devices and Environmental Modifications

- Use of **commode chairs, raised toilet seats**, incontinence pads/briefs.
- Ensure **clear path to bathroom, nightlights**, and **grab bars**.
- Provide **adaptive clothing** for easy removal.

V. Psychological and Social Impact

- Address **emotional distress**, embarrassment, and social withdrawal.
- Promote **support groups** or **psychological counseling** when needed.
- Encourage **patient dignity and privacy** during care.

VI. Cultural and Gender Considerations

- Recognize how **cultural norms** affect willingness to report or seek help.
- Respect **gender preferences** for caregivers, especially in intimate care.
- Use **trauma-informed approaches** for patients with abuse history.

VII. Evidence-Based Interventions

Add nursing interventions based on:

- **Agency for Healthcare Research and Quality (AHRQ)** recommendations.
- **National Institute for Health and Care Excellence (NICE)** guidelines.
- **Continence Nurses Society** or **International Continence Society** materials.